

Clinical Hypertension Management Guide

Hypertension is sustained blood pressure at or above 130/80 mmHg in adults. Initial evaluation includes repeated office measurements, home monitoring logs, and assessment of cardiovascular risk factors. Lifestyle modification is first-line: sodium reduction, DASH diet, weight management, and regular aerobic exercise.

Pharmacologic therapy may begin with thiazide diuretics, ACE inhibitors, ARBs, or calcium channel blockers depending on comorbidities. Diabetic patients often benefit from ARBs for renal protection. Monitor potassium and creatinine when initiating RAAS blockade.

Resistant hypertension requires review of adherence, white-coat effect, and secondary causes such as renal artery stenosis or primary aldosteronism. Ambulatory blood pressure monitoring clarifies nocturnal dipping patterns. Target organ damage screening includes carotid ultrasound and retinal exam.

Patient education covers medication timing, avoiding NSAID interactions, and recognizing symptoms of hypotension. Emergency referral is indicated for hypertensive emergencies. Follow-up visits should document home readings and adjust therapy incrementally.

Special populations: pregnant patients need methyldopa or labetalol rather than ACE inhibitors. Elderly patients may tolerate higher systolic targets to reduce falls risk. Comorbidities influence agent selection and dose titration schedules.